CHAI's RFI Response

Exec Summary:

CMS has established a leading national role in defining and enforcing clinical quality standards in the US, through programs like the Hospital Inpatient Quality Reporting Program, Hospital Value-Based Purchasing Program, the Merit-Based Incentive Payment Program, and its Five-Star Quality Rating System. Its quality measures, payment incentives, and reporting frameworks have influenced US payors and providers to embrace the shift to value-based care.

Meanwhile, AI can deliver groundbreaking clinical results. For example, an early-warning system for sepsis has cut hospital sepsis mortality by **18%**[1] and an AI-driven prior authorization platform automates up to 90% of prior authorizations and **reducing denials and getting patients care faster**[2].

At the same time, AI can play a transformative role in strengthening CMS's administrative capabilities. Building on the White House's recent memo, we encourage CMS to leverage AI to enhance payment integrity, modernize fraud, waste, and abuse (FWA) detection, and improve operational efficiency across Medicare and Medicaid. Rather than relying on retrospective investigations, AI can enable predictive, real-time oversight that reduces improper payments while minimizing provider burden and accelerating patient access to care. This would free up resources for CMS to focus on supporting rural and under-resourced health systems through innovative funding models.

CHAI's response urges CMS to catalyze this movements, and establish US health AI leadership in quality of care globally, by investing time and resources across five priority areas.

1. **CMS should invest in building patient trust** for adoption of AI solutions. Patients must feel confident in the care they receive, AI-enabled or otherwise. The right level of transparency in how AI is being used, privacy-preservation for the data used to test, train or validate that data, and patient education and AI literacy resources are critical to patient trust and consequent uptake.

2. CMS should **expand access to CMS' vast data resources** for developers and researchers. Opening de-identified claims and outcomes data to innovators will provide robust, feature-rich data, while conserving data privacy, to fuel AI innovation and subsequent clinical breakthroughs.

---

[1] https://www.medtechdive.com/news/johns-hopkins-sepsis-alert-reduced-deaths/627918/
[2] https://coherehealth.com/utilization-management-suite

3. We also urge CMS to **encourage data linkage** - for instance, enabling Medicare API data to be easily combined with EHR data - as well as facilitating interagency linkage with the FDA and NIH particularly. We believe empowering patients with access to their data as well as linking outcomes data with regulatory and research powerhouses, will give a more holistic, real-world view of how AI solutions are working, which will in turn translate to an eruption of health AI innovation.

4. Fourthly, CMS can accelerate provider adoption of AI by **directly integrating trusted AI tools and infrastructure into its value-based payment models and innovation programs**. By tying reimbursement to the use of validated AI that demonstrably improves outcomes or reduces burden, CMS can shift the market toward responsible innovation.

5. To achieve this, CMS should enable or establish **a scalable, real-world assurance ecosystem to ensure AI tools are trustworthy.** CMS could play more of a role in setting guidelines with industry and engagement with public-private partnerships to then encourage the **testing and validation of algorithms** against common standards so that only safe, effective, and bias-managed AI is deployed at scale.

These bold actions will **supercharge American health innovation** and **improve care quality, outcomes and cost**. This comment represents the best-available consensus of CHAI's Board and member workgroups as of June 2025. Please see the conclusion of this note for the makeup of our board and membership.

Section B: Patients and Caregivers

PC-5: Building Patient and Caregiver Trust and Engagement

Patients and caregivers will embrace AI when they trust the products. Our recommendation is for CMS to support frameworks and tools for transparency and real-world validation. CMS can cultivate a marketplace of AI that people trust and want to use by certifying what works and is safe, and by partnering with the community in AI literacy and education.

To encourage patient and caregiver interest in AI products, CMS should foster trust, awareness, and co-design. Key strategies include:

1. Trusted Product Certification: Work with stakeholders to establish a voluntary certification or "trustmark" for digital health AI tools that meet robust standards for quality, safety and efficacy. For example, CHAI's *Applied Model Card* framework serves

as a "nutrition label" for algorithms, disclosing a model's intended use, performance, and limitations. CMS could recognize third-party evaluations or require model transparency for any AI tool used in Medicare/Medicaid enrolled providers.

2. Patient and Caregiver Co-Design: Involve beneficiaries and caregivers in the development and testing of AI tools. CMS can fund human-centered design programs, for example, "entrepreneur-in-residence" initiatives where developers work alongside patients and caregivers in community settings. This approach builds trust and ensures the resulting products address real-world needs.
3. Digital Literacy and Outreach: Partner with community and convening organizations to educate patients and caregivers about available AI tools and how to use them safely.

PC-7: Evaluating Real-World Performance and Outcomes

Collecting real-world evidence will help CMS identify which innovations truly deliver value for patients. Real-world validation is a known challenge – currently, fewer than 2% of FDA-approved AI algorithms have been adopted widely, in part due to lack of proof they generalize beyond controlled settings. We emphasize that reliable real-world performance is critical and CHAI is committed to supporting the development of scalable validation tools to support this.

CMS should establish mechanisms to collect and learn from real-world data on AI solutions' impact on outcomes and costs. This could be done by creating a federated data network where developers and providers submit outcome data, or by integrating data collection into existing CMS programs.

1. Registry integration: In partnership with ASTP/ONC, CMS could stand up or integrate with an existing registry to gather standardized metrics on patient outcomes and model development. In order to receive an endorsement by CMS, health systems might commit to reporting de-identified performance data into this registry. To ease burden, automated data capture should be used whenever possible (for example, pulling claims or clinical data to evaluate outcomes, rather than manual reports).
2. Leverage Medicare Claims and Patient-Reported Data: CMS holds feature rich claims data that can indicate if an intervention is reducing utilization (e.g. fewer ER visits) or improving preventive care uptake. More could be done to link and integrate this into a more holistic view of an algorithms' performance.
3. Require Post-Market Monitoring in Procurement: If CMS (or Medicare Advantage plans) integrate third-party AI tools into care, require vendors to have a post-market surveillance plan – i.e. the vendor must monitor their product's ongoing accuracy, bias, and effectiveness in the real world and share those findings.

PC-9: Expanding Medicare Data Available via API

Increase the scope of CMS data accessible through APIs to fuel innovation for patient-facing products. High-value additions might include:

1. Medicare Advantage Data: Currently, Blue Button covers Fee-For-Service claims. CMS should work with Medicare Advantage plans to make encounter data and supplemental benefits data available via standardized APIs.
2. Provider Directory and Network Data: An API for provider networks, directories, and quality ratings would help AI care navigation tools guide patients to in-network, high-quality providers.
3. Patient Safety and Outcomes Data: As above, CMS could provide API access to de-identified comparative data such as hospital readmission rates.

In making more data available, privacy and security must remain paramount. CMS should ensure any new APIs require robust authorization so only the patient and their designated care managers access sensitive data. By adding more data to CMS's own APIs, CMS provides a standardized data source for innovators,which will stimulate developer interest as high quality training and validation datasets are so difficult to access.

Moreover, CMS should signal support for combining CMS data with other key data sources (e.g. VA, FDA, NIH and other networks) so a patient's various records can be unified. The more holistic the data environment, the more opportunity for AI solutions that truly help patients manage their health across settings.

Section C: Providers

PR-1: Incentivizing Provider Adoption of Trusted Digital & AI Tools

CMS should encourage healthcare providers, including those in rural or resource-limited settings, to leverage proven AI solutions by aligning incentives and reducing adoption risks.

Some key approaches:

1. Integrate into Payment Models: Incorporate the use of certified AI tools into value-based care programs and Medicare payment reforms. For example, CMS could align AI adoption with reimbursement structures so that safe, effective AI use is incentivized and financially rewarded.
2. Trust through Validation: Clinicians worry about recommending tools that might not be effective and that they are wholly liable for. CMS can mitigate this risk by curating a list

of trusted AI solutions. If CMS, alongside third-parties, can vet certain AI solutions for clinical quality, providers will be far more willing to use them.

3. Infrastructure Grants for Underserved Providers: Earmark funding to help rural clinics, critical access hospitals, and community health centers adopt needed technology.

Providers play a pivotal role in recommending and deploying AI to patients. By incentivizing adoption and smoothing implementation challenges, CMS can speed up the responsible use of the technology in all aspects of care.

PR-3: Ensuring Comprehensive Data Interoperability in Healthcare

It is critically important that all clinically relevant data in EHR systems be accessible for exchange, regardless of format, to support comprehensive, high-quality care and advanced analytics. CMS should work with ONC to achieve full spectrum interoperability, including structured and unstructured data. Key steps include:

1. Enable read/write API capabilities: EHR vendors and healthcare providers should not limit data sharing to only structured fields. CMS (through Conditions of Participation or payment rules) can require that providers make the entirety of a patient's record available via API upon authorized request. This includes clinical notes, PDF/scanned documents, imaging reports, and other unstructured content. Recent ONC rules already require an Electronic Health Information (EHI) export capability. We suggest CMS build on this by endorsing standardized API approaches for these data exports (instead of proprietary dumps).
2. Invest in Data Standardization/enrichment: We recommend CMS collaborate with industry on data standardization efforts. For example, develop open-source tools that convert narrative text into structured summaries or develop common data models that assimilate differential data sources into the same format to be more "AI-ready".
3. Balance Burden and Benefit: We acknowledge making *every* data format interoperable has cost and privacy implications. CMS should prioritize data that has clear patient care benefits, and sequence requirements guided by input on what data providers and patients find most valuable.

Incomplete interoperability hurts care quality and innovation. Providers in the field report difficulty accessing old records or external information when it's locked in faxes or PDFs – this fragmentation can lead to medical errors, repeated tests, or missed information.

PR-7: Reducing Provider Burden in Data Sharing and Quality Reporting

CMS can support providers in making high-quality data available for interoperability and quality improvement by requiring comprehensive data sharing to support greater transparency in quality measurement, and minimize redundant reporting burdens. CMS could mandate that all EHR data - structured, unstructured, and scanned - is made available for exchange and can be used for quality measurement, giving providers a clear line of sight into how their data is being used for reimbursement or payment.

1. "Enter Once, Use Many" for Quality Data: CMS should harmonize data definitions across programs so that capturing data once in standard fields supports care delivery, quality reporting, and research.
2. Provide Tools and Shared Infrastructure: For smaller or under-resourced providers, CMS should support national shared services, such as a centralized FHIR endpoint directory or simplified interfaces, so that connecting to networks and meeting data standards is seamless.

Reducing provider burden means embedding data exchange into the act of delivering care. CMS policies should ensure interoperability is intuitive, invisible, and beneficial.

Section E: Technology Vendors, Data Providers, and Networks

TD-1: Stimulating Innovation in the AI Ecosystem

CMS can foster a healthy vendor market by lowering entry barriers and signaling support for trustworthy innovation.

1. Define Priority Use Cases & Challenges: Clearly articulate high-impact problem areas where CMS wants to see tech innovation, and partner with NIH and FDA to create a clear cycle from research to innovation and regulation, to outcomes and reimbursement.
2. Increase Access to Data: Expanding safe and secure access to CMS data via robust APIs is foundational. Many startups struggle due to lack of real-world data for development and testing.
3. Regulatory Clarity and Pathways: Assure developers that if they build high-quality products, there is a pathway to adoption in CMS. This might include clearer guidance on reimbursement and modernized coverage approval processes.

A clear signal from CMS - that they are ready to pilot, adopt and reimburse effective AI solutions - will increase developer interest and innovation. Coupling that with resources (data, funding, expert guidance) will further catalyze the adoption of this innovation.

TD-2: Expanding Data Availability and APIs to Unleash Innovation

To unleash innovation, CMS should enhance its data offerings and API capabilities in collaboration with developers.

1. Broaden CMS API Data Content: Identify and prioritize high-value data not currently in Blue Button 2.0 and make it accessible. This includes things like Medicare Advantage encounters, Medicaid data for dually-eligible patients, and richer clinical data sources.
2. Encourage Data Linkage and Combination: CMS should not only provide its own data but also facilitate linkages – for instance, enabling Medicare API data to be easily combined with EHR data - and between government agencies (VA, FDA, NIH etc.).
3. Remove Obstacles to Data Access: common barriers include data use agreements that are too restrictive, high fees or costs for access, reticence from health systems/payors to share IP/assets, non-standardized and/or unstructured data, technical complexity in obtaining data, and unclear privacy guidance.

CMS controls one of the largest health datasets in the world. By making it more available, with all the privacy-preserving structures in place, CMS can exponentially improve care and reduce costs for its beneficiaries. Making more data available, including from underserved populations (through FQHCs, rural health centers, etc.), will lead to solutions that work for everyone.

[not-specific to an area, but broadly relevant to Section E] Building a Scalable Assurance Infrastructure for Trustworthy AI

Establish a scalable assurance infrastructure to evaluate and monitor health AI tools, ensuring they are safe, effective, and equitable for real-world use. We urge CMS to collaborate with HHS and external partners to create a public-private framework for AI oversight.

E.g.:

1. Federated Testing and Monitoring Network: Create a network of participating health systems and vendors that can train AI tools on diverse datasets and conduct ongoing monitoring. This could include sandboxes at institutions serving different populations (urban hospitals, rural clinics, VA, etc.) to validate that an AI model works across settings.
2. Certification or Reporting Program: CMS could require that any AI used in care for Medicare beneficiaries must produce a Model Card and undergo routine evaluation.
3. Continuous Post-Market Surveillance: Implement mechanisms for ongoing monitoring of AI performance in the field. This could involve collecting outcome data, user feedback, and even automated detection of anomalies or drift in AI outputs.

In responding to CMS's RFI, CHAI emphasizes a vision of responsible, human-centric innovation in AI. Our recommendations highlight the need to empower patients and caregivers with trustworthy tools, equip providers with interoperable data and incentives to embrace technology and catalyze value-based care progress, and establish infrastructure that assures the quality of AI as it scales.

CHAI and its members stand ready to support these efforts – from refining model card frameworks to participating in pilot payment programs – to ensure that innovation in AI truly translates to better health and wellbeing for patients and caregivers.

About CHAI

The Coalition for Health AI (CHAI) is an industry-led, non-profit public-private partnership that believes in the potential of Artificial Intelligence (AI) to advance the practice of medicine.

We bring together the broad spectrum of interdisciplinary stakeholders in the US health ecosystem to drive the development and appropriate use of Responsible AI in health. We create consensus among innovators, clinicians, health systems, payors, and expert stakeholders across the sector.

Our 4,000 members come from nearly 3,000 organizations, along with hundreds of individual experts and stakeholders actively participating in workgroups, bringing diverse perspectives to our community.

- **Health Systems:** Our community includes over 200 health system members, spanning regional providers like MedStar Health, Mercy, and Providence, as well as leading academic medical centers such as Duke Health, Mayo Clinic, and Stanford Medicine.
- **Professional Advocacy and Specialty Groups:** Core to our work is building on foundational knowledge and standards that ensure quality in health care. We have a strategic partnership with the National Health Council who help ensure patient and community advocacy groups lead our workgroups, and we have the President and CEO of the National Association of Community Health Centers on our board. Organizations like the American Society of Clinical Oncology, College of American Pathologists, National Council for Mental Wellbeing, HL7, the Patrick J. McGovern Foundation, and The SCAN Foundation are among more than 100 philanthropic, professional advocacy, medical specialty, and standards organizations that have joined the CHAI community.
- **Healthcare Industry:** We're thankful to have membership from leaders in the health industry from CVS and UnitedHealth Group to OCHIN and Solventum.
- **Startups:** 75% of members are from industry, of which 24% are startups with fewer than 50 employees. These startups are creating solutions used across healthcare. They include

Abridge, Aidoc, Ambience Healthcare, Affineon, Bayesian Health, Bend Health, Healthvana, Innovaccer, and Suki.

- **Tech Infrastructure Providers:** We are grateful for the broad participation of the AI frontier model and infrastructure providers, including OpenAI, Oracle, AWS, Google, Anthropic.

Board of Directors

- **Chair: John Halamka, M.D., M.S**., President, Mayo Clinic Platform
- **Secretary and Treasurer: Michael Pencina, Ph.D.,** Chief Data Scientist, Duke Health; Professor of Biostatistics & Bioinformatics and Director of Duke AI Health, Duke University
- **Suchi Saria, Ph.D.,** Endowed Chair & AI Professor, Johns Hopkins; Founder & President, Bayesian Health; Advisor, National Academy of Medicine AI Code of Conduct
- **Nigam Shah, MBBS, Ph.D.,** Chief Data Scientist, Stanford Health Care; Professor of Medicine and Biomedical Data Science, Stanford University School of Medicine
- **Eric Horvitz, M.D., Ph.D.,** Chief Scientific Officer, Microsoft; member, President's Council of Advisors on Science and Technology (PCAST)
- **Morgan Cheatham,** Partner, Breyer Capital; Medical Trainee, Brown University
- **Jennifer Goldsack, OLY,** CEO, Digital Medicine Society (DiMe)
- **Kyu Rhee MD, MPP**, President and CEO of National Association of Community Health Centres (NACHC)